

SESSION OBJECTIVES

1. Compare and contrast feeding and eating disorders as per Diagnostic and Statistical Manual of Mental Disorders, (DSMV) classification
2. Discuss the common behavioral and psychological features
3. Review the signs, symptoms, risk factors and subtle physical exam findings that may support each diagnosis
4. Utilize BMI calculations to aid in diagnosis of eating disorders
5. Identify common consequences and complications
6. Explain appropriate treatment options

Eating Disorders- Definition

- Eating disorders are mental illnesses characterized by disturbances in:
 - Body image
 - Weight
 - Dietary patterns



EATING DISORDERS -DSM 5

- The DSM 5 list of eating disorders
 - Anorexia nervosa
 - Bulimia nervosa
 - Binge eating disorder
 - Avoidant/restrictive food intake disorder
 - Pica
 - Rumination disorder
 - Other specified feeding or eating disorder
 - Unspecified feeding or eating disorder

ANOREXIA NERVOSA (AN)-DIAGNOSIS

- For diagnosis of AN the DSM V requires each of the following for a period of at least 3 months
 - Restriction of energy intake that leads to a low body weight (BMI<18.5)
 - Based on patient's age, sex, developmental trajectory, and physical health
 - Intense fear of gaining weight or becoming fat or persistent behavior that prevents weight gain
 - Despite being underweight
 - Distorted perception of body weight and shape ,extreme influence of weight and shape on self-worth or denial of the medical seriousness of one's low body weight

EATING DISORDERS

- Classified as mental illness but also have medical consequences
- Anorexia nervosa and Bulimia nervosa are among mental disorders associated with the highest mortality risk



ANOREXIA NERVOSA (AN)-SYMPTOMS

- Most common medical symptoms
 - Amenorrhea
 - Exertional fatigue
 - Weakness
 - Cold intolerance
 - Palpitations
 - Dizziness
 - Abdominal pain and bloating
 - Early satiety
 - Constipation
 - Swelling of the feet



ANOREXIA NERVOSA (AN)-COMPLICATIONS

- Refeeding Syndrome
- A potentially fatal condition caused by a rapid drop in serum phosphorous, magnesium, and potassium
 - Occurs with excessive reintroduction of calories
 - Particularly carbohydrates
- Associated with acute heart failure and neurologic symptoms



BULIMIA NERVOSA (BN)- DIAGNOSIS

- Diagnosis
 - Episodes of binge eating
 - Inappropriate compensatory behavior to prevent weight gain.
 - Binge eating and inappropriate compensatory behaviors occur, on average, at least once a week for three months.
 - The patient's self-evaluation is unduly influenced by body shape and weight.
 - The disturbance does not occur exclusively during episodes of anorexia nervosa
 - People can fall within the normal range for their weight

BULIMIA NERVOSA (BN)-DIAGNOSIS

- Inappropriate compensatory behavior
 - Self induced vomiting
 - Fasting
 - Misuse of laxatives, diuretics or other meds
 - Excessive exercising



BULIMIA NERVOSA (BN)-

- Binge Eating
 - Eating an unusually large amount of food in a discrete period of time (eg. 2 hours)
 - Patients feel that the eating cannot be controlled during the episode
 - Equivalent of 2 meals (2000 kcal)
 - Continues until the patient is painfully full or uncomfortable



BULIMIA NERVOSA (BN)-TREATMENT

- Treatment
 - Nutritional rehabilitation
 - Psychotherapy
 - Pharmacotherapy
 - Monitoring patients for medical complications



BINGE EATING

- **Binge eating disorder** is characterized by recurrent binge eating episodes during which a person feels a loss of control and marked distress over his or her eating. Unlike bulimia nervosa, binge eating episodes are not followed by purging, excessive exercise or fasting. As a result, people with binge eating disorder often are overweight or obese.

BINGE EATING DISORDER-DIAGNOSIS

- Diagnosis
 - Episodes of binge eating
- Binge eating episodes have at least three of the following:
 - Eating more rapidly than normal
 - Eating until feeling uncomfortably full
 - Eating large amounts of food when not feeling physically hungry
 - Eating alone because of embarrassment by the amount of food consumed
 - Feeling disgusted with oneself, depressed, or guilty after overeating
- Episodes occur, on average, at least once a week for three months.
- No regular use of inappropriate compensatory behaviors (eg, purging, fasting, or excessive exercise) as are seen in bulimia nervosa.
- Binge eating does not occur solely during the course of bulimia nervosa or anorexia nervosa.

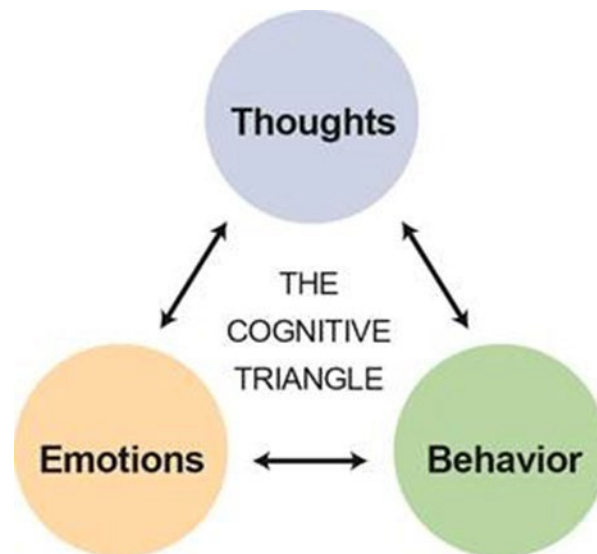
BINGE EATING DISORDER

- More prevalent than Anorexia or Bulimia
- Median age of onset is 21 years old
- Approximately 50 percent of individuals in the community with binge eating disorder are overweight or obese
- Female:Male is 2:1



COGNITIVE BEHAVIORAL THERAPY (CBT)

- Evidence-based treatment for psychiatric disorders
- Uses thought exercises or real experiences to facilitate symptom reduction and improved functioning



AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Onset usually occurs in infancy or early childhood and may persist into adulthood



AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Diagnosis
 - The eating behavior leads to a persistent failure to meet nutritional and/or energy needs, manifested by at least one of the following:
 - Clinically significant weight loss, or in children, poor growth or failure to achieve expected weight gain
 - Nutritional deficiency
 - Supplementary enteral feeding or oral nutritional supplements are required to provide adequate intake
 - Impaired psychosocial functioning

AVOIDANT/RESTRICTIVE FOOD INTAKE DISORDER

- Diagnosis Continued
 - The eating or feeding disturbance is not due to lack of food or a cultural practice.
 - The disturbance does not occur solely in the course of anorexia nervosa or bulimia nervosa, and body weight and shape are not distorted.
 - The disturbance is not due to a general medical condition or another mental disorder. When avoidant/restrictive food intake disorder occurs in the context of another illness, the eating disturbance is both out of proportion to what is expected for the other illness and warrants additional clinical attention.

PICA

- Diagnosis
 - Repeated eating of nonfood substances that are not nutritional, for at least one month.
 - The eating behavior is inappropriate to the patient's developmental level, and is not culturally supported or socially normal.
 - Minimum age of 2 years is suggested
- If the eating behavior occurs in the context of another mental disorder or general medical condition the severity of the eating behavior warrants additional clinical attention.

PICA

- Diagnosis
 - Repeated eating of **nonfood substances** that are not nutritional, for at least **one month**.
 - The eating behavior is **inappropriate to the patient's developmental level**, and is **not culturally supported or socially normal**.
 - Minimum age of 2 years is suggested
 - If the eating behavior occurs in the context of another mental disorder or general medical condition the severity of the eating behavior warrants additional clinical attention.

RUMINATION DISORDER-DIAGNOSIS

- Repeated regurgitation of food, which may be rechewed, reswallowed, or spit out
 - the eating disturbance occurs for at least one month.
- Regurgitation of food is not due to a general medical condition, such as gastroesophageal reflux disease or pyloric stenosis.
- Regurgitation does not occur solely during the course of avoidant/restrictive food intake disorder, anorexia nervosa, binge eating disorder, or bulimia nervosa.
- If the eating behavior occurs in the context of another mental disorder or general medical condition, the severity of the eating behavior warrants additional clinical attention.

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